



Basic Principles of Medicine II

Module: Nervous System & Behavioral Sciences

NB IMCQ 9

(Lifespan Development to Psychotherapies)

1) A 5-year-old girl is brought to the physician by her mother because of a 3-month history of unusual behaviors. Her mother says the child uses kitchen utensils and cardboard boxes to act out stories. She often pretends to be a teacher and can be overheard talking to imaginary students. At night, she is sometimes scared of monsters under her bed and asks that the bedroom light be left on. She is meeting age-appropriate developmental milestones and is otherwise healthy. Her uncle has a history of schizophrenia. Neuropsychiatric examination shows no auditory or visual hallucinations. The mother asks the physician whether she should be concerned about her daughter's behavior. Which of the following is the most appropriate response to address the parent's concerns?

- A. "This is normal for her age and reflects imaginative play and common childhood fears."
- B. "This behavior suggests an early psychotic disorder and requires urgent evaluation."
- C. "This behavior should be discouraged to prevent later social problems."
- D. "You should encourage your child to make real friends in school. This is not normal behavior."
- E. "Your daughter needs to be admitted for urgent psychiatric evaluation."

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2) A 4-year-old boy is brought to the physician by his mother because of a 2-week history of emotional distress. He has been visibly upset and withdrawn since the unexpected death of his grandfather. He continually asks, “When is grandpa coming back? Is he still angry with me?” He believes that he is the cause of his grandfather’s death because he was angry with his grandfather just before his passing. The mother is worried about the child’s emotional reaction to the passing of his grandfather. Which of the following is the most appropriate response by the physician?

- A. “This is a normal emotional response to death based on the child’s age.”
- B. “He seems depressed and requires immediate referral to a mental health specialist.”
- C. “I’m concerned that he blames himself for his grandfather’s death and recommend further evaluation for delays in cognitive development.”
- D. “His grief and emotional response are abnormal for his age, and discussing death with him should be avoided.”
- E. “His guilt and sadness indicate a significant trauma, and he should be immediately prescribed medication for depression.”

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3) A 28-year-old woman comes to the physician for a follow-up examination. She was recently diagnosed with a terminal illness and given a 2-month prognosis. During the encounter, she expresses profound regret and sadness. She feels bitter and angry that she will never marry, have children, travel, or do other things she dreamed of doing during her life. Which of the following characteristics is most likely associated with the successful resolution of her current psychosocial stage?

- A. Developing a sense of trust
- B. Developing a sense of independence and self-control
- C. Developing mastery and competency in skills
- D. Finding a sense of identity and role in life
- E. Forming close and meaningful relationships
- F. Feeling that you have contributed to the world
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4) A 20-year-old man comes to the physician for a follow-up examination. He has a 2-year history of mild depression and is receiving psychotherapy. He struggles academically in medical school, feels frustrated with his difficulties understanding concepts, and lacks confidence in his abilities. He recently got into an argument with his girlfriend after she concluded he was using projection as a defense mechanism. Which of the following statements by the patient most likely led to this conclusion?

- A. “I’m not going to think about my academic problems until after the holidays.”
- B. “I absolutely hate school, but I totally love you.”
- C. “It’s more important to have interesting life experiences than to do well in school.”
- D. “You think I am an academic failure!”
- E. “I am doing just fine in school.”
- F. “I am going to start a medical student support club.”

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5) A 72-year-old woman comes to the physician for a follow-up examination. She previously had laboratory and imaging studies done and is now receiving the results. The physician discloses that the results show advanced lung cancer with a 3-month prognosis. When she receives the information, she is shocked and in disbelief, saying that she feels perfectly fine and the results must be wrong. By the end of the encounter, the woman progresses to the next stage of grief. Which of the following statements is most likely reflective of her stage of grief at the end of the encounter?

- A. “There must be some mistake. I feel fine, so these results cannot be right.”
- B. “How could you have missed this earlier? This is your fault!”
- C. “If I try every possible treatment and do everything perfectly, maybe this can still be stopped.”
- D. “I do not see the point in anything anymore. I feel overwhelmed thinking about what is coming.”
- E. “I understand that time may be short, and I want to focus on being comfortable and with my family.”

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6) A 45-year-old woman comes to the physician for a follow-up examination. She recently underwent imaging studies and a biopsy because of a 3-month history of abdominal pain, nausea, vomiting, and weight loss. The physician discloses that the results confirm advanced pancreatic cancer with a poor prognosis. She becomes visibly distressed and says, “This can’t be possible. I eat healthy, I exercise every day, and I don’t drink alcohol or smoke. There must be a mistake.” Which of the following is the most appropriate initial response by the physician?

- A. “It is crucial that you recognize that your symptoms will only get worse. We need to begin discussing advance directives as soon as possible.”
- B. “I understand that this is difficult to hear. Would you like to talk about it or ask any questions?”
- C. “We can repeat the tests if that would reassure you. Would you like me to schedule an appointment for you?”
- D. “Let us focus on making you comfortable and discussing a future pain management plan.”
- E. “Pancreatic cancer can still occur in people with healthy lifestyles.”

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7) A 43-year-old man comes to the physician for a routine examination. He enjoys eating fast food and drinking carbonated beverages, despite knowing these habits are unhealthy. He has a 10-year history of obesity and hypertension. The physician recommends improving his diet by reducing his intake of processed foods and sugary drinks. The patient is considering these recommendations because he understands the importance of changing his lifestyle. However, he is concerned about whether he can sustain such a dietary regimen. Which of the following best describes the patient's behavior prior to his current stage of behavioral change?

- A. Evaluated the effectiveness of a healthy eating plan he had followed for more than 6 months
- B. Showed disinterest in starting any healthy diet plan
- C. Weighed the benefits and drawbacks of changing his eating habits
- D. Developed a specific plan to begin meal preparation and reduce sugary drinks tomorrow
- E. Followed a healthy eating plan for 1 week

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8) A 25-year-old woman comes to the physician 2 hours following a condom break during sexual intercourse. She requests a prescription for the emergency contraceptive pill. The physician agrees that her request is medically appropriate for her situation and that it is legally available via prescription. However, he tells her that prescribing the pill would violate his personal moral beliefs. Which of the following is the most appropriate action by the physician to resolve this ethical dilemma?

- A. Decline to prescribe the medication and instead counsel the patient about more reliable contraception for future use
- B. Invoke conscientious objection and refer the patient to another source of providers who will prescribe the medication
- C. Ask the woman to discuss with her partner before deciding whether to prescribe the medication
- D. Prescribe the medication because a physician must provide any legal and medically appropriate treatment requested by a competent patient
- E. Explain that prescribing emergency contraception conflicts with his beliefs and ask the patient to contact another physician on her own

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9) A 35-year-old man comes to the physician because of a 3-month history of worsening anxiety. His anxiety started after he was involved in a motor vehicle collision. Since the event, he has avoided driving because he fears becoming overwhelmed and losing control. The physician uses graded exposure therapy to treat the patient. Which of the following instructions is the most appropriate to give during the first exposure session of this therapy?

- A. “Start with a mildly anxiety-provoking driving situation and use progressive muscle relaxation.”
- B. “Start with a mildly anxiety-provoking driving situation and remain in it without using any safety behaviors.”
- C. “Immediately drive on a busy, unfamiliar highway and endure the anxiety.”
- D. “Challenge your thought that driving is dangerous by thinking about all the times you have safely driven a car.”
- E. “Think back to your childhood and how you felt when your parents were driving you around.”

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10) A 29-year-old man is brought to the physician by his wife for a follow-up examination. When he is asked about how he is feeling, he responds by saying, "They are destroying too many sheep just to make coats. If you go to buy gasoline, there will be no more of it. I want to smoke cigarettes, but the nurses will not let me." Which of the following best describes this patient's thought process?

- A. Pressured
- B. Circumstantial
- C. Tangential
- D. Flight of ideas
- E. Derailed
- F. Perseverative
- G. Ruminative

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11) A 38-year-old woman is brought to the emergency department by her husband because of a 1-month history of unusual behavior. She spends most of the day in her room with the blinds drawn and refuses to eat food she has not personally prepared, explaining that “people are trying to poison me.” During the interview, she alternates between tearfulness and laughter, sometimes within the same minute. When asked about her mood, she replies, “The neighbors keep sending signals through the walls. I’ve been thinking about painting again. My dog buried a dead bird in the yard.” She occasionally glances toward the empty corner of the room and whispers, “Don’t you see and hear them? They’re pointing and laughing at me.” Which of the following best describes this patient’s perceptions?

- A. Delusional
- B. Illusory
- C. Hallucinatory
- D. Labile
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12) A 16-year-old girl is brought to the physician by her mother because of a 2-month history of pain at the base of both thumbs. The symptoms started after she began working in a strawberry field picking berries off the vines. The physician diagnoses bilateral thumb tendonitis and recommends that she rest her hands for several days. She resists this advice because she would lose income. In addition to their regular salary, workers receive a bonus during field inspections if they are actively picking berries, rather than on a break. These inspections happen at unpredictable times throughout the week. Which of the following best describes the operant schedule in which the bonuses are applied?

- A. Fixed interval
- B. Variable interval
- C. Fixed ratio
- D. Variable ratio
- E. Continuous

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13) A 5-year-old boy is brought to the physician by his parents because of a 3-month history of behavioral problems. Every night after being put to bed, he repeatedly cries and calls out to his mother. To soothe him, the mother sits in his room and reads stories from his favorite books until he falls asleep. Since then, he has been calling out more often and for longer periods. The physician recommends that the mother stop going to the boy's room once he goes to bed, and explains that if she is consistent, the "calling out" behavior will eventually decrease. Which of the following learning principles most likely underlies the physician's recommendation on how to reduce the "calling out" behavior?

- A. Positive punishment
- B. Negative punishment
- C. Positive reinforcement
- D. Negative reinforcement
- E. Operant extinction
- F. Classical extinction
- G. Extinction burst

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14) A 30-year-old woman comes to the physician for a follow-up examination. She is receiving psychotherapy for anxiety that started 3 years ago at a company event. During that event, she was unexpectedly called upon to give a speech and stumbled over her words. Afterward, she overheard her colleagues making negative comments about her, which left her feeling intensely anxious. Since then, she gets anxious whenever there is a company event and has consistently avoided them to reduce her anxiety. Which of the following learning principles best explains the patient's continued avoidance of company events?

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15) A 35-year-old man comes to the physician because of a lifelong history of anxiety whenever he visits the dentist's office. As a child, he underwent reconstructive dental treatment that required several painful drilling procedures. Since then, he becomes extremely anxious whenever he hears the sound of a drill or when he is in the waiting room of the dentist's office. Which of the following learning theory concepts best describes the painful drilling procedures in this conditioning process?

- A. Positive punishment
- B. Negative punishment
- C. Unconditioned stimulus
- D. Unconditioned response
- E. Neutral stimulus
- F. Conditioned stimulus
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**We invite you to connect with us
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Please email us with any questions.

Visit us during office hours or email for an appointment.

